

Evidence review: alcohol and other drug (AOD) policy

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This review summarises international and national evidence on alcohol and other drug (AOD) abuse policies, to supplement community participation work on AOD abuse in the Wits/Agincourt research unit. The images to the right are drawn from this work (Mabetha, 2018; Oladeinde et al., 2019)

Alcohol and other drug (AOD) abuse is an issue of increasing public health importance globally. Alcohol consumption is associated with over 200 conditions and about three million people died as a result of its harmful use in 2016 (World Health Organization, 2011). In 2014, approximately 207,400 other drug-related deaths were recorded (UNODC, 2016). In sub Saharan Africa, alcohol abuse accounts for 6.4% of all deaths and 4.7% of all Disability Adjusted Life Years (DALYs), which is expected to rise in future (Ferreira-Borges et al., 2017).

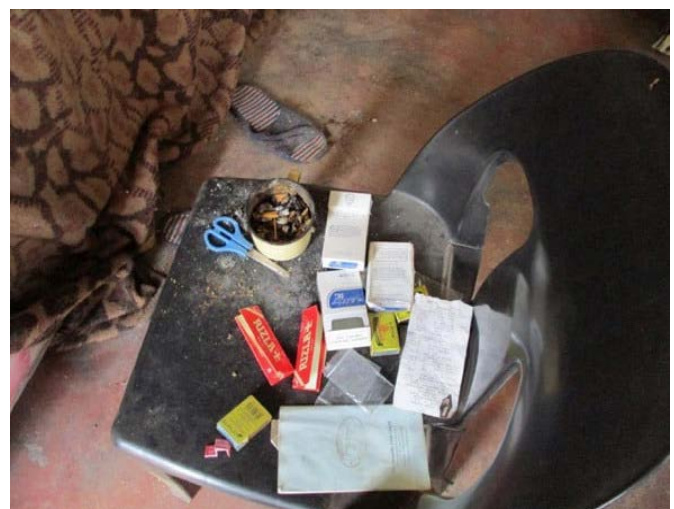
AOD abuse is a major challenge in South Africa. The end of the apartheid regime of institutionalised racial segregation in 1994 instituted a new political order promoting inclusive development to right past injustices. The health sector underwent fundamental transformation to a system focused on equitable provision, prevention and health promotion. With democracy, however, there was also an expansion of international trade links and improvements in air and sea travel. Combined with its geographical position, the availability and affordability of AODs increased sharply and rural, black communities, ravaged by poverty and racial segregation borne of decades of oppression, became conducive environments for abuse and dependency (Pasche, Sonja; Myers, 2012; Peltzer et al., 2010).

Drug use in South Africa is illegal and up to 15% of South Africans are reported to use drugs (INCB, 2015). Consumption rates are comparable to other regions, with harmful use ranked high, and about a third of adults who consume alcohol report harmful use (Morojele et al., 2006; Morojele and Ramsoomar, 2016; Pasche, Sonja; Myers, 2012; Puljević and Learmonth, 2014). HIV/AIDS has been causally linked to AOD abuse and associated with of risk-taking including crime and violence (Kang'ethe et al., 2017; Parry, 2010; Puljević and Learmonth, 2014)

Historically, AOD policies were characterised by a prohibitionist and supply-reduction focus, aimed at a realising drug-free society (Pienaar and Savic, 2016). The *National Drug Master Plan (NDMP)* of



An elderly woman lies down after drinking a number of bottles of beer. The photo was taken in the early hours of the day to show how widely alcohol use and abuse is accepted in the community: it is normal for anyone to drink alcohol at any time of day. Image credit: Ennie Khoza / VAPAR learning platform



Remnants of drugs smoked by one person. The high levels of addiction to marijuana and Nyaope (antiretrovirals that are mixed and smoked) is of great concern in the community because of the linkages to crime and abuse. Photographer: Florah Mafuyeka / VAPAR learning platform

2013-17 adopted a public health, rights-based, harm reduction approach, reflecting a transition to a more progressive, liberal, 'joined-up government' stance (DBE, 2013; DOH, 2017; DSD, 2013). Forward-thinking policies have, however, been rendered ineffective by various structural and organisational challenges including poor leadership, lack of personnel and funding (Otu, 2011; Shung King et al., 2014). In 2016, the Department of Planning, Monitoring and Evaluation reported no significant reduction in AOD abuse (DPME, 2016).

The changes in AOD policy acknowledge multiple, structural drivers that shape exposure, risk, behaviour and harms. While a view of *people-centred health systems* is advanced by WHO and there have been many calls to progress community participation to tackle entrenched health problems and realise equitable health (Meier et al., 2012; Mulumba et al., 2018; Prost et al., 2013; WHA, 2016) the concept remains poorly defined, especially at operational levels and communities grappling with AOD are rarely engaged in the development of responses (D'Angelo, Karen A; Her, 2019; Meier et al., 2012; Rifkin, 2014; Sondaal et al., 2018).

Post-apartheid, there have been radical advances towards community-based PHC and community participation, with important shifts towards a public health approach to AOD abuse recently (Central Drug Authority, 2013; DOH, 2011b, 2015, 2018; DOH and DBE, 2012; South African Government, 1996, 2003). Progressive policies and programmes face many challenges however: underinvestment, human resource crises, corruption, poor stewardship, and deepening social and health inequalities (Coovadia et al., 2009; McIntyre, 2012; World Bank, 2018). While community participation is mandated in many PHC strategies and policies (DOH, 2011a, 2016; DOH and DBE, 2012) it remains poorly understood and unevenly operationalised in practice.

Sources: Mabetha D (2018) 'It destroys families and communities': alcohol and drug abuse in rural South Africa. In: 5th Global Symposium on Health Systems Research, 2018. Available at: <http://healthsystemsresearch.org/hsr2018/photovoice/it-destroys-families-and-communities-alcohol-and-drug-abuse-in-rural-south-africa/> and Oladeinde O, Mabetha D, Twine R, et al. (2019) Community knowledge on action to reduce alcohol and other drug abuse in a rural South African setting: a participatory action research study. *Unpublished manuscript*.

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